

IDEATION PHASE

Define the Problem Statements

Medical Diagnosis Center by ServiceNow Service Portal

Date	06-10-2026	Project Type	ServiceNow Healthcare Service Management
Maximum Marks	2 Marks	Document Version	1.0

1. Customer Problem Statement Framework

A strong problem statement should be written from the customer's point of view. It should identify who is experiencing the problem, what outcome they are trying to achieve, what prevents that outcome, why the obstacle exists, and how the situation affects the customer or the organization.

For the Medical Diagnosis Center, the problem is examined from the patient perspective and the operational perspective so that the proposed ServiceNow solution addresses the complete healthcare-service lifecycle rather than only the request form.

I am...	I'm trying to...	But...	Because...	Which makes me...
Customer / role	Desired outcome	Obstacle	Underlying cause	Impact / feeling

2. Problem Discovery Context

Medical and diagnostic service requests can involve appointment requirements, patient information, diagnostic services, report availability, healthcare-team assignment, and communication with the patient. These requests require accurate information, controlled access, appropriate assignment, and clear communication.

The proposed project therefore focuses on the process problem: how to transform a potentially manual and fragmented healthcare-service interaction into a controlled, traceable ServiceNow lifecycle with standardized request data, appointment coordination, workflow automation, diagnostic fulfilment tasks, notifications, and status tracking.

3. Customer Problem Statements

PS-1 — Patient / Service Requester

I am...	I am a patient requesting a medical or diagnostic service.
I'm trying to...	I'm trying to request the required healthcare service quickly, provide the necessary information once, and know the current status of my appointment or diagnostic request.
But...	But healthcare requests may involve unclear service paths, repeated communication, manual appointment coordination, or limited visibility after submission.
Because...	Because patient requests, appointment information, service assignment, and status communication may be handled through separate manual steps.
Which makes me...	Which makes me feel uncertain about whether my request has reached the correct healthcare team and when the requested service will be completed.

PS-2 — Doctor / Healthcare Professional

I am...	I am a doctor or authorized healthcare professional responsible for reviewing and handling patient service requests.
I'm trying to...	I'm trying to review relevant patient service information and handle assigned requests efficiently.
But...	But requests may not consistently present the required information or arrive through a predictable assignment and workflow path.
Because...	Because request routing, appointment coordination, and task assignment can depend on manual communication.
Which makes me...	Which makes reviewing and coordinating patient services slower and increases the effort required to track pending work.

PS-3 — Laboratory / Diagnostic Team

I am...	I am a laboratory technician or diagnostic team member.
---------	---

I'm trying to...	I'm trying to receive complete, actionable diagnostic requests and process them efficiently.
But...	But incomplete information, manual assignment, repeated clarification, and unstructured requests can interrupt diagnostic service work.
Because...	Because patient request capture, diagnostic task creation, assignment, and communication may not be connected through one automated lifecycle.
Which makes me...	Which makes me spend time on coordination and information clarification instead of focusing on diagnostic service delivery.

PS-4 — Healthcare Administrator / ServiceNow Administrator

I am...	I am a healthcare administrator or ServiceNow administrator responsible for maintaining the service process and its configuration.
I'm trying to...	I'm trying to maintain a reliable, secure, and manageable healthcare service workflow.
But...	But manual process dependencies can increase support effort, configuration complexity, and the number of exceptions requiring attention.
Because...	Because patient requests, appointment handling, diagnostic tasks, notifications, and access activities may be distributed across separate manual steps.
Which makes me...	Which makes ongoing maintenance, service monitoring, and process governance more difficult.

4. Cross-Cutting Root Causes

Root Cause	How it contributes to the problem
Fragmented service entry	Patients may not always have one standardized path for different medical and diagnostic services.
Inconsistent request data	Missing or unclear patient/service information can create clarification cycles.
Manual appointment and routing	Requests may require human coordination before reaching the correct healthcare professional or diagnostic team.
Disconnected service steps	Patient information, healthcare tasks, appointment status, and communication may not be connected.
Limited lifecycle visibility	Patients and staff may need to ask for updates instead of seeing meaningful request status.
Governance dependency	Healthcare-related information requires controlled access, appropriate roles, approvals, and traceability.

5. Impact Chain

Incomplete / inconsistent request	→	Manual clarification	→	Delayed or less predictable service
Missing information	→	Repeated communication	→	Additional effort for healthcare team
Manual appointment/routing	→	Waiting / follow-up	→	Longer coordination cycle
Limited status visibility	→	Repeated status enquiries	→	Poorer patient experience

6. Consolidated Problem Statement

Patients need a simple and reliable way to raise medical and diagnostic service requests with complete information and clear status visibility, while doctors and healthcare teams need consistent information and structured work, and administrators need a controlled and traceable service process. Manual or fragmented healthcare-service handling can create incomplete data, coordination effort, appointment delays, repeated clarification, limited visibility, and additional operational workload.

Therefore, the project proposes a ServiceNow-based Medical Diagnosis Center solution that standardizes service capture through the Service Portal and Service Catalog, validates request information, coordinates appointments and assignments through defined workflows, orchestrates diagnostic/service tasks through Flow Designer, creates structured work for healthcare teams, protects access to healthcare-related information, and keeps patients informed through notifications and lifecycle tracking.

7. Problem-to-Solution Boundary

Problem Area	First-Release Response	Not Assumed in First Release
Service request	Service Catalog + guided form	Every possible healthcare service
Data quality	Mandatory / conditional validation	Fully autonomous clinical validation
Appointment / approvals	Controlled scheduling and assignment	Automatic approval of clinical decisions
Diagnostic fulfillment	Flow Designer + healthcare task	Uncontrolled automatic clinical actions
Communication	ServiceNow/email notifications	All external communication channels
Visibility	Request / task / appointment status tracking	Advanced predictive clinical analytics

8. Ideation Outcome

- Primary customer: Patient raising a medical or diagnostic service request.
- Operational customers: Doctors/healthcare professionals, laboratory/diagnostic team, healthcare support staff, and ServiceNow administrators.
- Core problem: Healthcare service requests require a more standardized, controlled, visible, and traceable lifecycle.
- Major pain points: inconsistent information, manual appointment/routing coordination, diagnostic fulfillment effort, limited visibility, repeated communication, and governance dependency.
- Proposed direction: a ServiceNow Service Portal workflow connecting Service Catalog, validation, appointment coordination, Flow Designer, healthcare tasks, notifications, role-based access, and tracking.

— End of Problem Definition —